

Revenue Integrity Scan: A TWMM Governance Framework

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Introduction

This comprehensive **Revenue Integrity Scan Guide** is designed to transform your clinic's financial "black box" into a transparent, governed system. By following this assessment, you can identify precisely where revenue is leaking and how to secure your practice's fiscal future.

1. Net Collection Rate (NCR) Analysis

- **Explanation:** NCR measures the percentage of "legally collectible" dollars your practice actually receives after all payer-contracted adjustments are made.
- **Importance:** It is the ultimate indicator of your billing team's efficiency and the accuracy of your cash flow.
- **Why Included:** A high Gross Collection Rate can mask deep inefficiencies if your adjustments are not being governed correctly.
- **Evaluation (Manual):**
 1. **Select Period:** Use a 90-day window to allow for claim aging.
 2. **Formula:** $NCR = (Total\ Payments \div (Total\ Charges - Contractual\ Adjustments)) \times 100$.
 3. **Target:** Aim for **96% or higher**.
- **Record:** Log the monthly NCR in a spreadsheet to track trends over time.

2. A/R Aging Velocity (0–30 to 90+ Days)

- **Explanation:** This tracks the percentage of total accounts receivable sitting in various time buckets.
- **Importance:** Money loses value the longer it stays in A/R; a ballooning "90+ Day" bucket often indicates unworked denials or timely filing risks.
- **Why Included:** Stagnant A/R is the leading cause of "ghost revenue"—money on the books that will never be collected.
- **Evaluation (Manual):**
 1. **Extract Report:** Pull an "A/R Aging by Payer" report from your EMR.
 2. **Benchmark:** Ensure **less than 15%** of your total A/R is in the 90+ day category.
- **Record:** Note the total dollar amount in the 90+ bucket and categorize it by top 3 payers.

3. Denial Root-Cause Identification

- **Explanation:** Categorizing why claims are rejected (e.g., Eligibility, Prior Authorization, Medical Necessity).
- **Importance:** Identifying the "Why" allows you to fix the process at the source, usually at the front desk or during clinical documentation.
- **Why Included:** Denials represent immediate revenue loss and increased administrative costs for re-billing.
- **Evaluation (Manual):**
 1. **Review EOBs:** Take a sample of 50 recent denials.
 2. **Tally:** Create a "Tally Sheet" for the top 5 reasons.
 3. **Target:** Maintain a Denial Rate of **less than 5%**.
- **Record:** Document the most frequent denial reason and the staff member or workflow responsible for that stage.

4. Level of Service (CPT) Frequency Distribution

- **Explanation:** Analyzing the distribution of E/M codes (e.g., 99213 vs. 99214) compared to specialty benchmarks.
- **Importance:** Under-coding leads to lost revenue, while over-coding creates significant audit risk.
- **Why Included:** Physicians often "default" to lower codes to avoid scrutiny, costing the practice thousands in legitimate revenue.
- **Evaluation (Manual):**
 1. **Report:** Pull a "CPT Frequency" report for each provider.
 2. **Analyze:** If a provider is billing 99213 for 90% of visits, review 10 charts to see if the medical decision-making (MDM) actually supported 99214.
- **Record:** Note any "Bell Curve" outliers among your provider team.

5. Missing Encounters Verification

- **Explanation:** Cross-referencing the appointment schedule against submitted claims to ensure every patient seen was actually billed.
- **Importance:** This identifies "leaks" where services were performed but a claim was never generated.
- **Why Included:** Even a 1% "Missing Encounter" rate can represent tens of thousands of dollars in lost annual income.
- **Evaluation (Manual):**
 1. **Compare:** Take one full day of the appointment schedule.
 2. **Verify:** Check each name against your "Claims Submitted" log.
- **Record:** Log any unbilled encounters and find the missing chart or documentation.

How TWMM Can Help You

We move beyond manual spreadsheets to provide high-level **Clinical Governance**.

- **Automated Governance:** We replace manual tallies with **PowerBI dashboards** that connect directly to your EMR/SQL data, giving you real-time visibility into these KPIs.
- **Process Remediation:** During the **Fix** phase, we don't just find the denials; we re-engineer your front-desk and clinical workflows to eliminate them.
- **Ongoing Monitoring:** We act as your external **Director of IT and Governance**, ensuring that as your practice grows in the **Tampa Bay** area, your revenue integrity remains at peak performance.

Ready to move from Assess to Fix? Contact us at admin@transworldmm.com or visit transworldmm.com to schedule your comprehensive Practice Health Assessment.